

Urinary Incontinence Primary Care Protocol

Key

- "Must do" actions for GP's / (Triaged by RMS)
- Recommendations for Primary Care
- Red flag / urgent referral
- Routine referral
- Public health intervention
- Audio-visual aids for patients and GP
- Click icon for clinical evidence

Female patient presents with urinary or bladder symptoms

Urgency, frequency and nocturia indicate an **overactive bladder**.
Loss of small amounts of urine associated with coughing, laughing, sneezing, exercising indicate **stress incontinence**.
Both of the above symptoms indicate a **mixed incontinence**

Examine abdomen and perform PV exam to exclude pelvic mass. BMI, avoid Genuine Stress Incontinence (GSI). Arrange US Pelvis if any doubt to exclude a mass.
Undertake a urine dipstick to detect presence of blood, glucose, protein, leucocytes and nitrites. Treat any infection with appropriate antibiotics.
Consider U/S KUB to measure post-void residue of volume in women with symptoms suggestive voiding dysfunction or recurrent UTI

Lifestyle Intervention

Caffeine – trial of caffeine reduction with over active bladder.
Fluid – advise modifications of high or low fluid intake in urinary continence or over active bladder
Weight – advise weight loss where BMI is >30

For Overactive and Urgency

Refer into Continence Service (community based) for bladder training for 6/52
Continence Service can prescribe medication if needed

Or

If Patient declines, start medical treatment

For stress incontinence

*If patient has **Mixed Incontinence** follow the arm for most predominant feature (if overactive bladder is more predominant follow the left arm, if stress incontinence is more predominant follow the right arm).*

Exercise Management

Offer Pelvic floor exercise in primary care + Bladder diary for 3 months

Refer to physiotherapy for pelvic floor exercise

Consider duloxetine second line only if women prefer pharmacological treatment or are not suitable for surgical treatment.

Medication Management

1st line: Immediate release oxybutynin (not frail/elderly) or tolterodine immediate release. Offer Mirabegron (if anti-cholinergics are contra-indicated)

2nd line: Solifenacin or Mirabegron if anti-cholinergic not tolerated or more than 2 anti-cholinergic ineffective.
Offer transdermal Oxybutynin to patients unable to take oral medication.

If post-menopausal, vaginal atrophy;

1st Line: Ovestin® cream

2nd Line: Vagifem® pessary

Use once daily for 14/7 then twice weekly

If troublesome Nocturia;

Consider desmopressin, 200mcg OD increasing to 400mcg if needed.

Avoid >65 with hypertension or CVD (see BNF for contra-indications and cautions)

If no improvement offer 3/12 refer to Gynaecology or Urology or Urogynaecology
Urology will see patients with a history of post bladder surgery, radiotherapy, complex pelvic surgery

Alternative Conservative Management

Consumables – absorbent products and toileting aids as a coping strategy (as per local CCG availability)

Catheters – Consider in patients with persistent urinary retention causing incontinence, symptomatic infections and renal dysfunction (as per local CCG availability)

Comments & enquiries relating to medication:

CCG Medicines Management Team: mmt.islington@nhs.net

Refer to current BNF or SPC for full medicines information

Clinical contact for pathway queries: Dr Karen Sennett

karen.sennett@nhs.net

Pathway provided by NCL & modified by Islington CCG in 2018.

Review date: July 2019

Reference – NICE Clinical Guideline CG171

Incontinence support services available to patients

Version 10.6 April 2018
Review Date: April 2019

SUPPORT	SUPPORT TYPE	TARGET GROUP	ACCESS	SUPPORT
CCG Community Continence and Stoma service Provider Barnet: CLCH District Nurses Camden: Whittington Community Bladder & Bowel Service Enfield: BEH-MHT Haringey: Whittington Community Bladder & Bowel Service Islington: Whittington Community Bladder & Bowel Service	Community Healthcare	All	Referral	It offers a comprehensive range of support to help people self-manage and remain independent, or to adapt and modify their lifestyles to adjust to increasing dependence. Housebound patients are seen by district nurses in the first instance. Help with catheter changes and continence pads, etc. <i>Barnet: CLCH District Nurses - 0845 389 0940 / 0203 209 7920</i> <i>Camden: Whittington Community Bladder & Bowel Service - 020 3316 8401*</i> <i>Enfield: 020 8702 5820**</i> <i>Haringey: Whittington Community Bladder & Bowel Service - 020 3316 8398*</i> <i>Islington: Whittington Community Bladder & Bowel Service - 020 3316 8401*</i> <i>*https://www.whittington.nhs.uk/default.asp?c=10154</i> <i>**http://www.beh-mht.nhs.uk/enfield-community-services/ecs-services/continence-service.htm</i>
Age UK	Web-based information	Older people	Any	Age UK have pages dedicated to information about incontinence https://www.ageuk.org.uk/Documents/EN-GB/Information-guides/AgeUKIG15_Managing_incontinence.inf.pdf?dtrk=true They also have produced a guide on common bladder and bowel problems and how to manage them. https://www.ageuk.org.uk/Documents/EN-GB/Information-guides/AgeUKIG15_Managing_incontinence.inf.pdf?dtrk=true
NHS Choices	Web-based information	All	Any	With more than 48 million visits per month, It is the UK's biggest health website, accounting for a quarter of all health-related web traffic. Has pages explaining incontinence conditions, and how to access services.
The Chartered Society of Physiotherapy	Professional Body	Pregnant women	Any	The Chartered Society of Physiotherapy (CSP) has lots of useful information about pregnancy-related incontinence. You can also use the CSP's Physio2U service to find a local practitioner or contact the Pelvic Obstetric and Gynaecological Physiotherapy group.
Squeezy	App	Women	Any	Squeezy helps women with their pelvic floor muscle exercises as part of a physiotherapy programme.
Squeezy for men	App	Men	Any	For men to manage their pelvic floor muscle exercises
Tät	App	Women	Any	Tät is a training program for your pelvic floor. It guides you through progressively challenging exercises to build up strength; when you master one, you move on to the next. And for each exercise, graphics illustrate how long, and how intensely, you should contract your muscles. The app also offers lifestyle advice, and lets you set reminders so you stick to your regular "workouts."
Bladder & Bowel UK	Web-based information	All	Any	Bladder and Bowel UK offer specialist clinical and product advice, support and practical help. We have a team of Specialist Nurses and Continence Product Information staff, who can be contacted on the National Confidential Bladder and Bowel UK help line (0161 607 8219) or via email at bladderandboweluk@disabledliving.co.uk
ERIC (Childrens bladder and bowel charity)	Helpline Web-based information	Children	Children	Charity dedicated to the bowel and bladder health of all children and teenagers in the UK. Our vision is that every child and teenager with a bowel or bladder condition can access support and live free from embarrassment, shame, isolation and fear.
Bladder & Bowel Community	Support website Online Forum Carers information and advice	All	Any	The Bladder and Bowel Community (B&BC) is the UK wide service for people with bladder and bowel control problems. B&BC provides information and support services, including a confidential helpline, for anyone affected by these conditions as well as their families, carers and healthcare professionals.
Prostate Cancer UK	Web-based information Helpline Online chat with specialist nurse	Men with prostate cancer	Any	Website has a section on Urinary problems after prostate cancer treatment. https://prostatecanceruk.org/prostate-information/living-with-prostate-cancer/urinary-problems
General Practice	Clinical	All	Any	Can provide treatment options including prescribed medicines
Midwives and Health Visitors	Community	Pregnant and Post-natal women	Pregnant and Post-natal women	What UI support is given by midwives during pregnancy? What UI support is given by health visitors?
Carers UK	Web-based information and advice helpline	Carers	Any	Our expert telephone advice and support service is here if you want to talk about caring. If you're looking for answers, our online information and support is the best place to start.
Barnet Carers Centre	Carers centre	Carers	Any	The Centre offers advice, information, emotional and practical support for all informal carers who live or work in the London Borough of Barnet.
Association for continence advice	Professional body	Health professionals	Health professionals	Multi disciplinary group of health care professionals providing information about all aspects of incontinence. Continence adviser provides telephone advice and answers letters.

Introduction

Many men may experience urinary leakage caused by a variety of problems and conditions. Some also have difficulty controlling wind or leakage from the bowels. This may be due to weakness of the muscles of the pelvic floor, which have an important function in preventing these troublesome symptoms.

What is Pelvic Floor?

The floor of the pelvis is made up of layers of muscle and other tissues. These layers stretch from the tailbone at the back to the pubic bone in front. A man's pelvic floor supports the bladder and the bowel. The urethra (bladder outlet) and the rectum (back passage) pass through the pelvic floor. The pelvic floor muscles play an important role in bladder and bowel control. The pelvic floor also plays a role in maintaining an erection.

Why the Pelvic Floor Muscles get weak

The pelvic floor muscles can be weakened by:

- Surgery for an enlarged prostate gland
- Continual straining to empty the bowels, usually due to constipation
- Persistent heavy lifting
- A chronic cough, such as smoker's cough or chronic bronchitis and asthma
- Being overweight
- Lack of general fitness

Neurological damage (e.g. after a stroke or spinal injury, or resulting from other neurological conditions such as multiple sclerosis) can also result in poor pelvic floor muscle function. People in this group should seek advice from a healthcare professional.

Pelvic Floor exercises

You can improve control of your bladder and bowel by doing exercises to strengthen your pelvic floor muscles. These exercises may also be used in conjunction with a bladder training programme aimed at improving bladder function in people who experience the urgent need to pass urine frequently and who may not always "make it in time". Bladder training is explained in B&BC's booklet 'A Healthy Bladder'.

How to contract the Pelvic Floor Muscles

The first thing to do is to correctly identify the muscles that need to be exercised.

1. Sit comfortably or lay on your bed with the muscles of your thighs, buttocks and abdomen relaxed.
2. Tighten the ring of muscle around the back passage as if you are trying to control diarrhoea or wind. You should be able to feel the muscle move. Don't try too hard otherwise you will start to squeeze your buttocks, thighs and/or your tummy muscles inappropriately.
3. Now imagine trying to draw your penis inside your body to shorten it, whilst at the same time lifting your scrotum upwards towards your tummy button. If your technique is correct, each time that you tighten your pelvic floor muscles you may feel a dip at the base of your penis and scrotum move up slightly.

If you are unable to feel a definite squeeze and lift action of your pelvic floor muscles, you should seek professional help to get your pelvic floor muscles working correctly - see the final section of this factsheet. Even men with very weak pelvic floor muscles can be taught these exercises by a physiotherapist or continence advisor with expertise in this area.

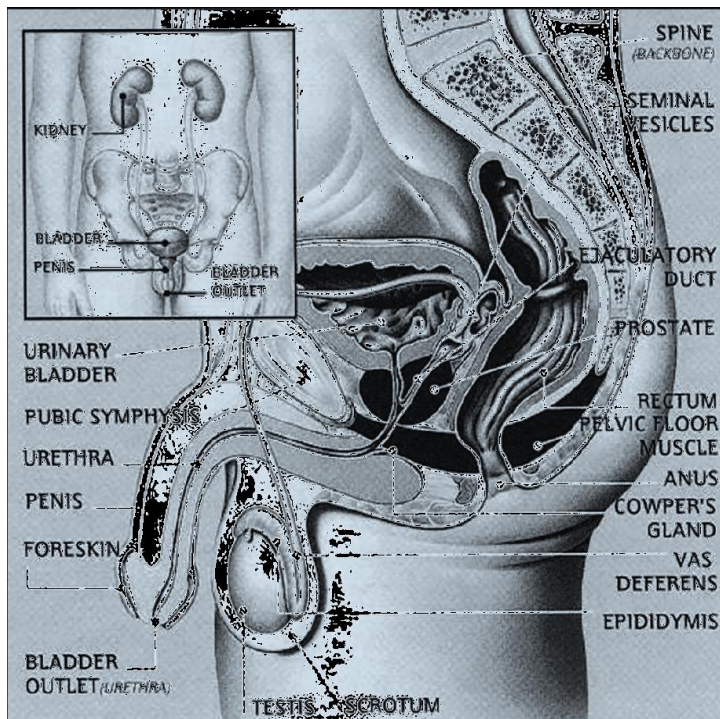
Doing Pelvic Floor exercises

Now you can find your pelvic floor muscles, here are the exercises to do:

1. Your pelvic floor muscles need to have stamina. To train for improved endurance sit stand or lie with your knees slightly apart. Gradually tighten and pull in the pelvic floor muscles. Keep shortening and lifting them for as long as you can. Rest for 4 seconds and then repeat the contraction. Build up your ability and strength until you can do 10 slow contractions at a time, holding them for 10 seconds each with rests of 4 seconds in between.
2. Your pelvic floor muscles also need to react quickly to sudden stresses from coughing, laughing or exercise that puts pressure on the bladder. Practice some quick contractions, drawing in the pelvic floor quickly and strongly and then holding it for a few seconds before relaxing. If you are sneezing or coughing, you need to be able to squeeze quickly as well as holding onto the squeeze until the sneeze or cough has subsided.

Aim to do a set of both slow contractions (exercise 1) and quick contractions (exercise 2) at least 3 times each day.

It takes time for exercise to improve muscles function. You are unlikely to notice any improvement for several weeks, so stick at it! You will need to exercise regularly for at least 3 months before the muscles gain their full strength.



Make the exercises a daily routine

Once you have learnt how to do these exercises, they should be done regularly, giving each set your full attention. It may be helpful to have regular times during the day for doing the exercises; for example, after going to the toilet, when having a drink, when lying in bed.

Also you need to tighten your pelvic floor muscles while you are getting up from a chair, coughing or lifting. Some men find that tightening before they do such things helps them to regain control.

Remember good results take time. In order to rehabilitate your pelvic floor muscles you will need to work hard at these exercises. You may not notice an improvement for several weeks and you will not reach your maximum performance for a few months.

When you have recovered control of your bladder or bowel you should continue doing the exercises twice a day for life.

Other tips to help your Pelvic Floor

- Share the lifting of heavy loads
- Avoid constipation and prevent any straining during a bowel movement
- Seek medical advice for hay fever, asthma and bronchitis to reduce sneezing and coughing
- Keep your weight within the right range for your height and age

Seeking help

To achieve your best results you may need to seek help from a specialist physiotherapist or continence advisor. They will have a range of treatments available that can help you learn how to improve your pelvic floor muscles functions.

You can get the details of your nearest Continence Clinic by phoning B&BC or visiting our website. Our contact details are below.

Prostate problems

B&BC also has a fact sheet on 'Your Prostate and You' which gives information and advice on prostate concerns

Bladder and Bowel Community

The Bladder and Bowel Community
7 The Court, Holywell Business Park, Northfield Road
Southam, CV47 0FS
General enquiries: 01926 357220
Email: help@bladderaandbowelfoundation.org
Web: www.bladderaandbowelfoundation.org

Introduction

Physiotherapists, doctors and nurses know that exercising the pelvic floor muscles can help you to improve your bladder control. When done correctly, these exercises can build up and strengthen these muscles and so help you to control your bladder and bowel.

What is a pelvic floor?

The pelvic floor consists of layers of muscle and ligaments that stretch from the pubic bone to the end of the backbone (coccyx) and from side to side (see diagram). Firm, supportive pelvic floor muscles help support the bladder, womb and bowel, and to close the bladder outlet and back passage.

How does pelvic floor work?

The muscles of the pelvic floor are kept firm and slightly tense to stop leakage of urine from the bladder and wind or faeces from the bowel. When you pass water or have a bowel motion the pelvic floor muscles relax. Afterwards, they tighten again to restore control. The muscles actively squeeze when you laugh, cough, lift or sneeze to help prevent any leakage. They also have an important sexual function, helping to increase sexual awareness for both yourself and your partner during intercourse.

How can exercising the pelvic floor muscles help?

Exercising the pelvic floor muscles can strengthen them so they give the correct support. This will improve your bladder control and improve or stop leakage of urine. Like any other muscles in the body, the more you use and exercise them, the stronger the pelvic floor muscles will be.

Finding your pelvic floor muscles

It is not always easy to find your pelvic floor muscles. Exercising them should not show at all 'on the outside'.

You should not pull in your tummy excessively, squeeze your legs together, tighten your buttocks or hold your breath!

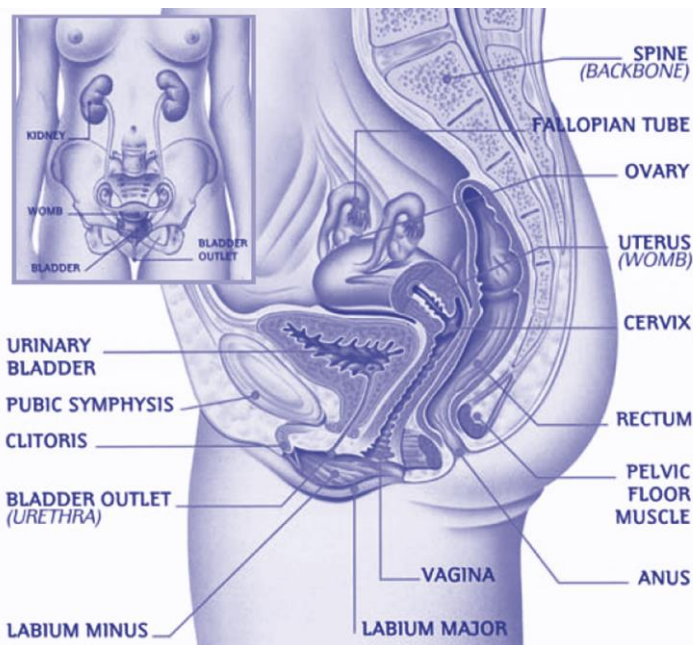
Here is what to do:

1. Sit comfortably with your knees slightly apart. Now imagine that you are trying to stop yourself passing wind from the bowel. To do this you must squeeze the muscles around the back passage. Try squeezing and lifting that muscle as if you really do have wind. You should be able to feel the muscle move. Your buttocks and legs should not move at all. You should be aware of the skin around the back passage tightening and being pulled up and away from your chair. Really try to feel this squeezing and lifting.
2. Now imagine you are sitting on the toilet passing urine. Picture yourself trying to stop the stream of urine. You should be using the same group of muscles that you used before, but don't be surprised if you find this harder. (Do not try to stop the stream when you are actually passing water as this may - if repeated - cause problems with correct emptying).
3. Now try to tighten the muscles around your back passage, vagina and front passage and lift up inside as if trying to stop passing wind and urine at the same time. It is very easy to bring other incorrect muscles into play, so try to isolate your pelvic floor as much as possible by not squeezing your legs together, not tightening your buttocks and not holding your breath. The lower tummy can very gently be drawn in as if pulling away from the zip of tight trousers. In this way most of the effort should be coming from the pelvic floor muscles.

Practising your exercises

Now you can find your pelvic floor muscles, here are the exercises to do:

1. Your pelvic floor muscles need to have stamina. So sit, stand or lie with your knees slightly apart. Slowly tighten and pull up the pelvic floor muscles as hard as you can. Try lifting and squeezing them as long as you can. Rest for 4 seconds and then repeat the contraction. Build up your strength until you can do 10 slow contractions at a time, holding them for 10 seconds each with rests of 4 seconds in between.
 2. Your pelvic floor muscles also need to react quickly to sudden stresses from coughing, laughing or exercise that puts pressure on the bladder. So practise some quick contractions, drawing in the pelvic floor and holding it for just one second before relaxing. Try to achieve a strong muscle tightening with up to ten quick contractions in succession.
- Aim to do a set of slow contractions (exercise 1) followed by a set of quick contractions (exercise 2) 3-4 times each day.
- It takes time for exercise to make muscles stronger. You are unlikely to notice any improvement for several weeks - so stick at it! You will need to exercise regularly for at least 3 months before the muscles gain their full strength.



Tips to help you

1. Get into the habit of doing your exercises during normal day to day activities. For example, whilst cleaning your teeth or waiting for a kettle to boil.
 2. If you are unsure that you are exercising the right muscles, put your thumb into the vagina and try the exercises to check. You should feel a gentle squeeze as the pelvic floor muscle contracts.
 3. Tighten your pelvic floor muscles when you feel you might be about to leak - pull up the muscles before you cough, laugh, sneeze or lift anything heavy. Your control will gradually improve.
 4. Drink normally - about 6-8 large glasses of fluid a day, avoiding caffeine if you can. Water is best! And don't get into the habit of going to the toilet 'just in case'. Go only when you feel your bladder is full.
 5. Watch your weight - extra weight puts extra strain on your pelvic floor muscles.
 6. Once you have regained control of your bladder, don't forget your pelvic floor muscles. Continue to do your pelvic floor exercises a few times each day to ensure that the problem does not come back.
- Remember: you can exercise your pelvic floor muscles wherever you are - nobody will know what you are doing!**

Do you have any questions?

This information sheet is designed to teach you how to control your bladder, so that you'll be dry and comfortable. If you have problems doing the exercises, or if you don't understand any part of this information sheet, ask your doctor, nurse, continence advisor or specialist continence physiotherapist for help. Do your pelvic floor muscle exercises every single day. Have faith in them. You should begin to see results in a few weeks if you are exercising correctly - but don't stop then: make the exercises a permanent part of your daily life.

Pelvic floor exercises for men

Men have the same sling of pelvic floor muscles as women, and if they have 'weak bladders' (particularly after treatment for an enlarged prostate) they too can benefit from pelvic floor muscle exercises.

Bladder and Bowel Foundation

SATRA Innovation Park
Rockingham Road
Kettering
Northants NN16 9JH
General enquiries: 01536 533255
Nurse helpline: 0845 345 0165
Counsellor helpline: 0870 770 3246
Email: info@bladderandbowelfoundation.org
Web: www.bladderandbowelfoundation.org

Date: I woke up at: I went to sleep at:

Time	Record drinks (type and amount)	Each time you use the toilet to pass urine (please tick below)	When you changed a pad/ panty liner (please tick below)	Each time you leak urine, circle whether you were:			
12am				Almost Dry	Damp	Wet	Soaked
1am				Almost Dry	Damp	Wet	Soaked
2am				Almost Dry	Damp	Wet	Soaked
3am				Almost Dry	Damp	Wet	Soaked
4am				Almost Dry	Damp	Wet	Soaked
5am				Almost Dry	Damp	Wet	Soaked
6am				Almost Dry	Damp	Wet	Soaked
7am				Almost Dry	Damp	Wet	Soaked
8am				Almost Dry	Damp	Wet	Soaked
9am				Almost Dry	Damp	Wet	Soaked
10am				Almost Dry	Damp	Wet	Soaked
11am				Almost Dry	Damp	Wet	Soaked
12pm				Almost Dry	Damp	Wet	Soaked
1pm				Almost Dry	Damp	Wet	Soaked
2pm				Almost Dry	Damp	Wet	Soaked
3pm				Almost Dry	Damp	Wet	Soaked
4pm				Almost Dry	Damp	Wet	Soaked
5pm				Almost Dry	Damp	Wet	Soaked
6pm				Almost Dry	Damp	Wet	Soaked
7pm				Almost Dry	Damp	Wet	Soaked
8pm				Almost Dry	Damp	Wet	Soaked
9pm				Almost Dry	Damp	Wet	Soaked
10pm				Almost Dry	Damp	Wet	Soaked
11pm				Almost Dry	Damp	Wet	Soaked